

DISCHARGE SUMMARY

PATIENT INFORMATION

Name: Patient 23

Age: 55 years old | Sex: Male

MRN: 000-TEST-23

Admission Date: [Date of Admission]

Discharge Date: [Date of Discharge]

Length of Stay: 3 days

=== PRIMARY & SECONDARY DIAGNOSES ===

1. Type 2 Diabetes Mellitus with hyperglycemic episode
2. Hyperlipidemia
3. Hypertension (controlled)

HOSPITAL COURSE

Patient 23 presented to Metro Medical Center via ED with complaint of polydipsia, polyuria, and fatigue × 5 days. Admission labs revealed glucose 387 mg/dL, HbA1c 9.2% (baseline ~8.1%), consistent with acute hyperglycemic decompensation. No evidence of DKA or HHS. Urinalysis showed glycosuria 3+, no ketonuria. Patient was transitioned from home metformin monotherapy to insulin + oral agent regimen given severity of presentation. Continuous glucose monitoring (CGM) device placed on hospital day 2. Patient educated extensively on carbohydrate counting, sick day management, and hypoglycemia recognition. Fasting glucose improved to 156–178 mg/dL by discharge. No acute complications noted; EKG and troponin serial negative. Lipid panel reviewed; simvastatin dose optimized.

MEDICATIONS AT DISCHARGE

Medication	Dose	Frequency	Duration
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Insulin glargine (Lantus)	20 units	q.d. @ bedtime	Ongoing
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Insulin lispro (Humalog)	8–12 units	AC meals + correction scale	Ongoing
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Metformin	1000 mg	BID	Ongoing
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| Simvastatin | 40 mg | q.d. @ evening | Ongoing |

| Lisinopril | 10 mg | q.d. | Ongoing |

| Aspirin | 81 mg | q.d. | Ongoing |

GLUCOSE MONITORING SCHEDULE

- Capillary fingerstick glucose (FSG) QID: fasting, pre-lunch, pre-dinner, bedtime
- CGM worn continuously; review trends weekly
- Home A1C check in 12 weeks

ACTIVITY & RESTRICTIONS

- No heavy lifting or strenuous exercise × 1 week; resume normal activity as tolerated
- Continue daily ambulation; standing tolerance as per baseline
- Avoid prolonged sitting; encourage frequent position changes

DIETARY RECOMMENDATIONS

- ADA diabetic diet: consistent carbohydrate distribution across meals
- Target: 45–60g carbohydrate per meal, 15–20g per snack
- Limit simple sugars, refined carbs; emphasize whole grains & legumes
- Sodium restriction <2g/day (hypertension management)
- Fluid intake: 2L/day unless contraindicated

FOOT CARE INSTRUCTIONS

- Daily foot inspection (dorsal/plantar surfaces, between toes, heels) for breaks, erythema, calluses
- Avoid going barefoot; wear protective footwear at all times
- Trim toenails straight across; do NOT use razors or chemical removers
- Report any foot pain, numbness, open sores, or color changes IMMEDIATELY
- Monofilament testing to assess sensation; if diminished, refer to podiatry

FOLLOW-UP APPOINTMENTS

1. Endocrinology clinic: in 2 weeks (insulin titration & glucose log review)
2. Primary Care Physician: in 1 week (medication reconciliation, BP check)
3. Diabetes education (RN or RD): in 3 weeks (reinforce carb counting, sick day protocol)

